AirSense 10 CPAP Machine

TECHNICAL WRITING CASE STUDY | STANDARD OPERATING PROCEDURE

Deliverable	Audience	Status
Four-page SOP + technician record	Trained hospital technicians	Draft portfolio exemplar

The documentation problem

Returned CPAP equipment involves several distinct tasks: identifying the machine, checking for damage, separating returned components, cleaning permitted external surfaces, and recording the result. A single vague “clean and return” instruction would not explain component-specific routing or when a technician must stop work.

Audience and objective

I designed the procedure for trained hospital technicians, including staff who are new to the return workflow. The objective was to make the sequence easy to follow, make exceptions visible, and prevent an exterior-cleaning label from being mistaken for authorization to reuse the machine.

Writing and design decisions

- **Task-based sequence.** Organized the procedure from receipt and identification through component routing, exterior cleaning, and final recording.
- **Controlled terminology.** Used consistent identifiers for work areas (AREA-01–04), labels (LBL-001–003), records (FRM-001–002), and the cleaning-method reference (REF-03).
- **Visible exceptions.** Separated electrical warnings, PPE requirements, stop-work actions, and release restrictions into distinct color-coded panels.
- **Component routing.** Used a table to distinguish reusable, disposable, and separately assessed items rather than treating every returned component alike.
- **Plain procedural language.** Used short, action-led steps informed by ASD-STE100 principles; no claim of formal ASD-STE100 compliance is made.

Outcome

The result is a four-page draft SOP with a component-routing table, numbered technician actions, stop-work instructions, and a completion record. It demonstrates procedural writing, information architecture, safety communication, and document-control thinking.

Source and validation boundary

The ResMed Air10 clinical reprocessing addendum informed the distinction between exterior cleaning and component reprocessing. Product selection, contact time, local work areas, disposal routes, and release authorization require manufacturer and hospital verification. The sample is explicitly marked DRAFT — NOT FOR CLINICAL USE.